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Disrupting Health Professions Education Research: A Guide to Critical Reflexive Praxis during Research Planning

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ABSTRACT

Research is never truly neutral; all research is inherently subjective because it is shaped by who conducts it, how they think, and the systems they operate within. Paradoxically, despite reflexivity's critical intent – a practice for recognizing and addressing researcher influence – it too often becomes a superficial checkbox exercise that fails to meaningfully challenge deeper structural and systemic inequities. As a result, Health Professions Education (HPE) research often reinforces global power imbalances, privileging Western perspectives while excluding knowledge from the Global South and marginalized communities. This article advocates for Critical Reflexive Praxis (CRP), an approach grounded in Critical Theory, that combines self-reflection with deliberate action to disrupt power dynamics and promote equity in research. CRP extends beyond traditional reflexivity by interrogating and transforming the underlying structures that shape knowledge production and dominant research practice. By adopting CRP, researchers can challenge entrenched hierarchies, include and amplify marginalized perspectives, and create research that fosters meaningful social transformation. This article offers practical guidelines for enacting CRP across individual, interpersonal, local, and global levels during HPE research planning, paving the way for more equitable and impactful contributions to HPE and beyond.

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Introduction

All research, whether qualitative or quantitative,^{1,2} is inherently subjective because it is shaped by researchers' perspectives, choices, and interpretations. From the formulation of research questions to the selection of methodologies, data analysis, and interpretation of findings, researchers' assumptions and biases are ever present and influential. This reflects what is called the “socially constructed” nature of knowledge: what we consider to be “true” or important is shaped by people, cultures, and societies rather than being “purely” and “objectively” discovered like a natural law. This lack of neutrality is true for educational research in medicine and the health professions for the research theories and practices (“praxis”) that dominate and are considered “good” is influenced by global systems of power that are largely structured along a North-South divide.^{3–5} To illustrate, health professions education (HPE) scholarly output (e.g.,

publication numbers) from the global North (i.e., USA, UK, Canada, Netherlands, Australia) continues to far outstrip output from the global South.^{6,7}

While there are myriad reasons for this North-South divide (pragmatically: prohibitive publication costs, lack of funding, viewed as contextual outsiders, lack of global North scholars as coauthors who are considered “legitimate”; academically: forced to publish in English, and use of non-“normative” theories and methodologies),^{6,8,9} many of them are rooted in colonial histories. This is why modern medicine, and related HPE research and scholarship, can be called “colonial artifacts.”^{10–12} For instance, colonizers imposed “universal” and “objective” Western medical knowledge on Indigenous peoples, often dismissing and erasing local healing traditions, which is seen today in the continued domination of positivist perspectives,^{13,14} various scientific theories,¹⁵ evidence-based practice models,¹⁶ the biomedical model,¹⁷ and an assumption that “West is

best,”^{10,18} which all continue to frame HPE and its research globally.

The exclusion of knowledge from the global South can be termed epistemological gatekeeping¹⁹ or epistemic violence⁶ where ideas from these settings are denied or silenced. Yet why should the global HPE field care? An unequal knowledge landscape is problematic because it limits diverse participation and inclusion, leading to incomplete and biased knowledge and evidentiary bases, with “best” practice recommendations produced that are not relevant nor appropriate, thereby hindering innovation and progression, and resulting in continued harmful and discriminatory practices, with negative implications for research, education, and health outcomes.^{4,20} HPE researchers therefore need to take the “geopolitics” of knowledge and research into account, as well as acknowledge and examine their own “epistemic locations” – meaning that they are critically aware of where and how knowledge is produced, whose knowledge is valued, how research can reinforce or challenge power structures, and their role in these processes.¹¹ We argue that this can be achieved through critical reflexive praxis (CRP).

Too often, researchers engage in reflexivity, “the act of examining one’s own assumption, belief, and judgement systems, and thinking carefully and critically about how these influence the research process” (p.1),²¹ in a superficial manner. For example, after including a perfunctory positionality statement, researchers will then continue to conform to hegemonic (dominant Western) research philosophies and practices, and fail to realize the impact of their research in the world in (unintentionally) upholding knowledge hierarchies.^{22,23} CRP calls for researchers to be cognizant of their positions within the global knowledge hierarchy.^{4,24} Furthermore, CRP differs from reflexivity in that it extends from basic self-reflection and critical awareness to an *action-orientation* that strives to disrupt and transform current structures and systems of inequity.^{25,26}

The aim of CRP is to close the North-South divide and equalize the research landscape. This aim aligns with recent calls for “epistemic reflexivity,” “social reflexivity,”^{19,27} and “emancipatory knowledge” (i.e., knowledge that brings about new and creative ways of thinking to realize social transformation).²⁷ Researchers should therefore question and disrupt the “intangible social processes that influence what theories inform a discipline’s knowledge base... [and] the invisible assumptions that pervade everyday theorising and practice.”¹⁹ CRP further relates to calls for *resistance*.^{5,28} Resistance has been defined as something that is *active*: it is affirmative (fighting for something),

deliberate (intentional), proportionate (reasonable), and constructive (productive).²⁸ This means CRP adopts both deconstructive *and* constructive stances: *disrupting* discrimination, oppression, and marginalization in research *to rebuild* in equitable ways that promote equality in research for all.²⁸ Additionally, CRP supports *anti-racist praxis*,²⁹ which recognizes that we can perpetuate harm if we do not take deliberate action against it.

Now that the unequal and exclusionary state of HPE research has been outlined, along with the limitations of traditional reflexivity, we present what a critical theoretical perspective is, what it adds to CRP and thus why it is necessary. This is followed by guidance on enacting CRP, focusing on the initial stages of conceptualizing and designing of a research project.

Author positionality

Consistent with the notion of critical reflexivity, we wish to position ourselves within the research and broader HPE realm. DS is a White South African female who is working in the global North, grappling with identity dissonance and tension. Her lived experiences in a “post”-colonial and “post”-Apartheid setting critically conscientised her to issues of power and her unconscious complicity. As a non-health professional, and also a nonsocial scientist, but a basic science-trained researcher who jumped into education, she adopts insider/outsider perspectives to problematize “norms.” Her agenda as an educationalist is social and epistemic justice. As an HPE researcher from the global South, DS has experienced exclusion in the field, as have many of her colleagues; yet now, working in the global North with increased participatory access, she is using this power to include those from the global South – prioritizing collaborations with Southern scholars, conducting research projects that benefit the global South, and through adopting decolonial approaches to her work.³⁰ PS is a proud Biripi-settler man, Community member, and father from the global South, who also identifies as a medically trained Aboriginal Health academic. His lived experience as an Aboriginal-settler man and health professions academic within a colonized nation has enabled him to view his context through the applied Mi’kmaw concept of “two-eyed seeing.” This unique lens has been instrumental in identifying both limitations and value in diverse ways of knowing, being, and doing between Aboriginal and Western structures and systems, including within a research context. As an Aboriginal academic, his approach to research has been and is Community-centered. He is critically

conscious of his accountability to his Community and the responsibility he maintains to utilize his position to actively promote Community sovereignty over research that involves or impacts his Community. While our paths and lived experiences are diverse, they have crossed at a crucial time for our global society, one in which the need for CRP has never been greater.

The necessity for a critical theoretical perspective

CRP is rooted in a critical theoretical perspective because it moves reflexivity from introspection alone, to requiring researchers to *act on* their reflections in ways that promote equity and justice. Briefly, Critical Theory (CT) (Figure 1) is an umbrella term that encompasses a body of theoretical work and sets of theories (e.g., feminist theories, Critical Race Theory, anti-colonial theories, intersectionality, Marxist theories, queer theories, Critical Disability Theory, etc.) that examine and challenge unfair social structures and systems of power.³¹ A critical framework highlights and explains that these inequalities exist because certain ways of thinking, knowing, and acting have been given more importance over time, while others have been ignored or suppressed.³² Ontologically (i.e., views on the nature of reality), CT views reality as a phenomenon that is shaped by social, political, cultural, and economic power structures over time.³¹ Epistemologically (i.e., views on the nature of knowledge), CT recognizes that knowledge is influenced by social and historical backgrounds, meaning some perspectives are seen as more valid than others.³¹

Axiologically (i.e., views on what is seen as valuable, worthwhile, ethical), CT promotes fairness and equality by encouraging diverse perspectives and questioning dominant power structures.¹⁶ Indeed, if CRP is understood as, not just part of rigorous research practice, but *ethical* research practice,^{33,34} then there are imperatives for “beneficence,” “non-maleficence,” and equity – aligning to CT’s values of democracy and egalitarianism. Methodologically (i.e., views on how to do research), CT values different experiences and voices, and participatory approaches, helping to uncover and address hidden inequalities.¹⁶ Importantly, CRP inherently attends to the interconnected concepts of ontology, epistemology, axiology, and methodology, as they pertain to research practice. A critical theoretical perspective enables more effective reflexive praxis for it identifies targets for meaningful reflection and action that might otherwise go unexamined. CT expands the scope of reflexivity beyond individual behaviors and immediate contexts to include broader and deeper taken-for-granted power-laden societal systems and structures.^{13,31}

The need for critical reflexive praxis in research

Traditionally, reflexivity has focused on reflection at a personal level, such as considering one’s preconceptions, beliefs, values, multiple identities, and social positions. However, researchers often fail to dive deeper to examine the underlying systems and structures that result in the power and privilege, or disadvantage and discrimination, they possess, and that have shaped their preconceptions, beliefs, and values.³⁵

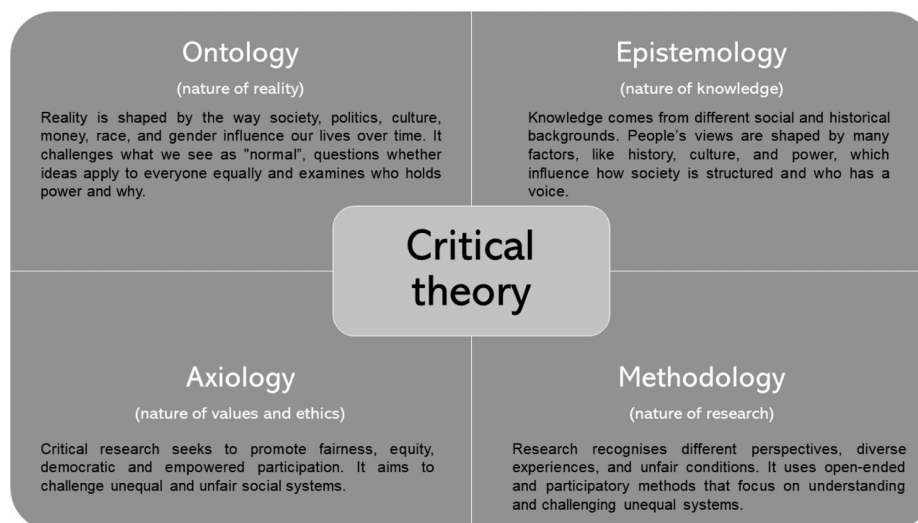


Figure 1. An overview of a critical theoretical perspective.

For example, failing to see the need to reflect on taken-for-granted characteristics, such as race, gender identity, sexual orientation, culture, language, class, religion, age, ability, employment status, nationality, and citizenship status, reveals an inherent privilege (power) and reflexive blind spot. This reveals a lack of consideration for greater systems and structures of inequity, such as White supremacy, patriarchy, heteronormativity, and coloniality, for example.^{35,36} This underscores a word of caution: confessions of privilege are meaningless if not accompanied by tangible action. These confessions can be harmful if used for a loophole or absolution of guilt; moreover, they can re-centre Whiteness as a way to assert authority and legitimacy (and thus reinforce unequal power relations).^{37,38}

A guide for critical reflexive praxis in health professions education research

HPE researchers should engage in CRP throughout the research process, however, before embarking on a new research project, extensive groundwork must first take place. It can be helpful to reflect and act on several “levels:” the individual or personal, interpersonal, immediate (local) context and broader field (global context). Such work should focus ontological (e.g., HPE assumptions around society, including how and why they exist), epistemological (e.g., assumptions around HPE knowledge production, including who holds knowledge and its value), methodological (e.g., assumptions on how HPE research should be undertaken) and axiological issues (e.g., assumptions around why certain HPE research is undertaken and valued, and others not) (Table 1). Think of a stone thrown into a pool of water, and its outward ripples of ever-growing circles: individuals and research do not exist in isolation but impact on self, others and broader realities.

Importantly, there is no set list of actions for researchers to adopt to engage in CRP, in fact, performative CRP is often counterproductive to CRP's intent. Indeed, in considering the diversity of researchers, contexts, and projects, proclaiming a single or universal approach to CRP is contradictory – there will be immense variation, messiness and nuance needed while figuring it out. Rather, while not exhaustive nor prescriptive, we offer broad guiding principles with examples of reflective prompts and possible actions to stimulate researchers' own developmental journeys toward *becoming* and *being* more critically reflexive – as this work is ongoing.³⁹

A key example of CRP applied to HPE research relates to the “hidden curriculum,”⁴⁰ a concealed acculturation into a specific health discipline and broader health system that perpetuates epistemic and power inequities.⁴¹ Critically examining the impacts of the “hidden curriculum” in HPE has fostered a greater awareness regarding how positive outcomes can be promoted, while mitigating negative impacts.^{42,43} Furthermore, critical reflexive research centering underrepresented voices has revealed the inequitable and harmful outcomes of the hidden curriculum on marginalized patient groups.^{44,45} Given the hidden curriculum's intent of professional acculturation *via* a hegemonic value system, it is of little surprise that marginalized patients, who occupy non-“normative” ways of knowing, being, and doing, fare worse than those whose values are aligned with the same prevailing system.

Individual level

Traditional reflexivity is often practiced at the individual level, without considering the systems and structures they operate within. Therefore, while these guiding principles may be familiar to readers,²² we encourage a deep and honest interrogation when reflecting on personal biases, assumptions, and privileges – and what larger factors may have shaped these – while progressing to broader levels of CRP. Acknowledge how your intersectionality influences how you think about and practice HPE research.

Perhaps the simplest, yet most powerful (and potentially uncomfortable), reflective prompt to ask is: what kind of researcher does one want to be? This requires first being honest about the type of researcher one currently is, then reimagining the kind of researcher one *could be* – which is transformative and reflects the developmental nature of CRP.³⁹ CRP is necessary for researchers who value diversity, equity, justice, and belonging for all in HPE.⁴⁶ Overarching principles are to strive for humility, be critically self-aware and willing to hold yourself accountable to un-learn and re-learn if necessary.

CRP at an individual level can be aided by maintaining a reflexive journal, having critical dialogues with others, practicing reflexivity collaboratively,^{22,47} and revising research beliefs and behaviors over time.

Interpersonal level

For interpersonal CRP, guiding principles include relationship building with stakeholders and affected

Table 1. Reflective prompts and example actions for critical reflexive praxis at varying levels during the initial stages of an HPE-focussed research project.

	Individual	Interpersonal	Local context	Global context
Reflective prompts	- How does my background shape my assumptions and biases? - How does my identity and positionality influence my understanding of the research topic? - What privileges might blind me to alternative perspectives? - What theories and methodologies do I use and value – and why? - How does my choice of statistical models or analytical methods reflect my biases or assumptions? - Have I considered whether my quantitative measures adequately represent diverse perspectives? - Are my data sources representative of the population I intend to study?	- Who holds power in this research setting, and how does this influence the work? - Whose perspective is absent, ignored or excluded? - What are my assumptions and biases about potential participants and co-researchers? - Who does this research benefit or harm? - How am I ensuring respect and reciprocity with others? - Is the language I am using accessible? - Am I ensuring that co-researchers or assistants have access to data literacy training?	- What are my assumptions and biases about the research context? - Am I a contextual insider or outsider? - How does this context impact the research and vice versa? - Should I be leading this research project? - What is the potential impact of my findings on the local community, particularly in terms of representation or policy implications? - Does my quantitative data collection process account for local cultural norms or practices? - Are the instruments and surveys I use validated for this specific context?	- Where am I located in the global knowledge hierarchy? - Does this research disrupt or reinforce global knowledge hierarchies? - Who do/don't I collaborate with and why? - Am I challenging global power hierarchies and amplifying excluded perspectives? - Does my dataset reinforce global inequities, such as over-representation of certain populations or geographies? - Am I using statistical methods that prioritize generalizability without ignoring local variability?
Example actions	- Keep a reflexive journal. - Seek outside feedback on blind spots. - Revise assumptions regularly. - If appropriate, draw on critical theories and alternative, participatory methodologies when designing your research project. - Test assumptions of analysis models, methods, and techniques for alignment with diverse population dynamics. - Consider using mixed methods (as opposed to quantitative alone) to balance quantitative findings with qualitative insights.	- Diversify research teams. - Meaningfully engage stakeholders and local communities affected by the research phenomenon (i.e., potential participants). - Engage in dialogues, actively listen, adjust based on diverse inputs and mutual benefits, and (re)centre community perspectives. - Respect and promote self-determination. - Undertake research projects that align with community needs and priorities. - Practice reflexivity collaboratively. - Avoid academic jargon or technical terms. - Train team members in inclusive data handling and analysis techniques. - Foster open discussions about methodological choices and their potential biases in qualitative and quantitative research. - Collaboratively develop decision rules for data exclusions, interpretations, or transformations.	- Share power and partner with local community members. - Treat locals as experts (i.e., avoid tokenistic inclusion). - Be humble and adopt the position of learner. - If appropriate and necessary, be prepared to not lead the research project. - Plan for contextually and culturally appropriate and participatory research designs. - Share resources and funding with partners. - Regularly revisit the relevance and cultural validity of measurement tools. - Develop outreach materials (e.g., reports, infographics) that explain quantitative findings in accessible terms relevant to the local audience.	- Read and cite excluded researchers (e.g., those from the global South and marginalized communities). - Collaborate with excluded researchers. - Listen actively and respectfully. - Advocate for equitable research resourcing and funding. - Push for systemic change in institutional or organizational support for diverse epistemologies and methodologies. - Seek out and incorporate datasets from underrepresented regions to balance global perspectives. - Use hierarchical or multi-level modeling to capture global-local interactions in the data.
Worked example	- Reflect on how the hidden curriculum may afford oneself privileges, if at all. - Critically examine how the hidden curriculum shapes one's assumptions, beliefs, biases and "normative" or "ideal" practices. - Identify how implicit messages in training reinforce dominant norms and exclude alternative perspectives in one's practices.	- Identify and challenge how one's relationships with colleagues and/or trainees could be creating hierarchical relationships and reinforcing power inequities. - Pay attention to and address how informal learning in research teams may unintentionally perpetuate exclusionary norms in one's practice.	- Reflect on the assumptions the hidden curriculum makes about certain contexts and groups of people. - Address the hidden curriculum's impact on marginalized groups by incorporating underrepresented perspectives in (re)design and transformation. - Examine how local training and mentorship structures reinforce or disrupt inequitable power dynamics.	- Identify the social, historical, political, economic, etc. factors that have influenced the development, spread and reproduction of the hidden curriculum. - Examine and challenge how global health education, policies and regulation reinforce epistemic and power hierarchies through the hidden curriculum. - Investigate how funding priorities shape research questions and the invisibility of marginalized perspectives.

Note: for mixed methods studies, reflective prompts and actions should be drawn from both qualitative and quantitative recommendation.

communities based on respect, trust, accountability, and reciprocity, while being aware of and mitigating unequal power dynamics. This means reflecting on what relationships (or not) one has with participants and the contexts within which research takes place. Reflect on who has been included, and who has been excluded, marginalized or silenced; whose voice is centered;³⁹ and what you have done to encourage diversity and inclusivity in your research?

When reflecting on what counts as knowledge,³¹ a critical perspective embraces and values *all* voices – with participants viewed as *experts* to work *with* and not objects to extract data *from*.^{11,35} This can be supported through meaningfully engaging with stakeholders and communities prior to undertaking research, to first understand *their* self-determined needs and priorities (and to counter misconceptions, discriminatory stereotypes, or prejudices one may hold about them), and then to *collaboratively* make decisions and *co-design* projects that are mutually beneficial.^{11,46,48} This further supports CRP at a personal level, because it forces researchers to examine their underlying motivations and relationships to a particular research topic^{39,49} and the types of questions asked.¹⁰

Practically, once a diverse research team has been established and power distributed evenly across the team, consider the prospect of regular team debriefings that explicitly discuss uncomfortable issues like power, expectations, roles, and co-construct agreements on how to operate.^{49,50} Actively listen to, understand, and uphold team members' and participants' right to self-determination.³¹ Overall, prioritize social accountability and collective ownership over autonomy and personal gain.^{30,38,48,51}

Local contextual level

Overlapping with interpersonal CRP, guiding principles for contextual CRP include grounding research in specific social, cultural, economic, and historical contexts, integrating local knowledges and practices, and acknowledging the potential impact of the research on local communities.

Ask yourself how well you understand the cultural, social, historical, political, and economic context of the research setting. Reflect on whether you are an insider or an outsider to the context in which the research is conducted. This could impact on whether you lead a particular research team or not^{10,27,47} – instead it may be more appropriate to be a “guest” partner in some contexts.⁵² Stakeholder and community engagement and collaboration can inform contextual grounding of research, and development of an

appreciation for potentially differing cultures (i.e., beliefs, values, traditions, norms and practices) that helps to minimize harm to local communities. This does not merely relate to the research topic (i.e., one that benefits the local context first and foremost), but in *how* the research is conducted (e.g., participatory, culturally-appropriate, accessible – non-exploitative or extractive – practices).^{30,31,38,51}

Global contextual level

Ultimately, CRP is about being critically conscious, which, in a research context, involves recognizing the local and global power dynamics that impact on research agendas and practices, and then acting to redress such forces to realize more equal knowledge systems. Ask yourself: how does your research engage with global power hierarchies in HPE; what role do you (unintentionally) play in perpetuating or disrupting them; are you amplifying marginalized voices and perspectives in global HPE knowledge production; and does your work align with global HPE priorities for equity and justice? For example, you may have a favorite theorist, author, or preferred methodology you use by default; instead, consider interrogating whether these reflect deeper misconceptions and prejudices, which could risk maintaining discriminatory hegemonies. In other words, the use of dominant educational theories and methodologies are not neutral, but can implicitly reproduce White, Western, and colonial norms that maintain epistemic oppression and reinforce the global HPE knowledge hierarchy.^{10–12,53} For instance, a false preconception is the belief that Indigenous approaches, critical theories,^{10,31,47} or nontraditional and “alternative” methodologies, like participatory approaches,^{48,49,54} are illegitimate or inferior to “normative” research designs, theories, and practices. CRP demands transparent methodological decision-making and an openness to using alternative paradigms, that promote ontological, epistemological, axiological, and methodological diversity and transformative outputs within HPE research projects.^{19,55}

We call all researchers to challenge epistemic inequities and strive to balance representation from the Global North and Global South, Indigenous and non-Indigenous perspectives, and those who identify with underserved and marginalized communities. One way this can be done is through reading, citing, and collaborating with underrepresented (*excluded*) researchers.⁵⁵ Relatedly, advocate for equitable research funding and capacity-building initiatives that benefit the Global South, diverse Indigenous communities,

and other marginalized groups, while also committing to resource sharing. Additionally, it may mean undertaking research beyond what institutions fund.⁵¹

Conclusion

Being confronted with the realities of global HPE knowledge hierarchies can be daunting and uncomfortable – but ignoring them allows these inequities to persist. While there are many valid approaches to tackling this challenge, we focussed on the initial stages of planning an HPE research project as a critically important first step to dealing with epistemic injustice. CRP is not just an intellectual exercise or a methodological consideration; it is a transformative commitment to equity, justice, and inclusivity in HPE research. Rooted in CT, CRP challenges researchers to move beyond introspection to action-oriented praxis, to disrupt and dismantle entrenched power structures, to contribute to a more equal HPE knowledge landscape. This approach calls for deeper engagement with the systems and contexts that shape HPE research, from individual biases to global hierarchies. By going beyond surface-level reflexivity, CRP pushes researchers to critically examine who holds power in knowledge production, whose voices are missing, and how research can become a tool for equity rather than exclusion. By embracing CRP, researchers in HPE can create work that does more than merely meet academic standards – it can drive meaningful societal change. CRP must be viewed not as a static practice but as an ongoing, collective responsibility to interrogate and rebuild research practices in ways that are ethical, inclusive, and transformative. Ultimately, adopting CRP demands humility, civil courage,⁵⁶ and a commitment to lifelong un-learning and re-learning. It requires researchers to ask difficult questions, engage in critical dialogue, and take actionable steps (even small ones – which are still meaningful and impactful) toward change. Yet, note that this is a developmental journey that requires researchers be gracious and patient with themselves, as change is often slow and incremental. Transforming HPE does not require perfection, but intentional, ongoing effort. Moreover, it can be encouraging to know that one is not alone, and that *collective* CRP is powerful enough to disrupt and reimagine HPE systems and structures. Together, as scholar-activists, we must continually strive to create research that not only reflects diverse ways of knowing, being, and doing but also contributes to a more just world.

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References

1. Garcia NM, López N, Vélez VN. QuantCrit: rectifying quantitative methods through critical race theory. *Race Ethn Edu*. 2018;21(2):149–157. doi:10.1080/13613324.2017.1377675.
2. Garcia NM, López N, Vélez VN. *QuantCrit: An Antiracist Quantitative Approach to Educational Inquiry*. 1st ed. London: Routledge; 2023.
3. Naidu T, Ramani S. Transforming global health professions education for sustainability. *Med Educ*. 2024;58(1):129–135. doi:10.1111/medu.15149.
4. Sims D. When I say ... global south and global north. *Med Educ*. 2024;58(3):286–287. doi:10.1111/medu.15263.
5. Rogers LO, Moffitt U, McLean KC, et al. Research as resistance: naming and dismantling the master narrative of “good” science. *Am Psychol*. 2024;79(4):484–496. doi:10.1037/amp0001246.
6. Naidu T. The personal is political in the struggle for equity in global medical education research and scholarship. *Med Teach*. 2023;45(9):991–996. doi:10.1080/0142159X.2023.2206535.
7. Maggio LA, Costello JA, Ninkov AB, et al. The voices of medical education scholarship: describing the published landscape. *Med Educ*. 2023;57(3):280–289. doi:10.1111/medu.14959.
8. Mokhachane M, Green-Thompson L, Wyatt TR. Voices of silence: experiences in disseminating scholarship as a global south researcher. *Teach Learn Med*. 2024;36(2): 235–243.
9. Naidu T, Cartmill C, Swanepoel S, et al. Shapeshifters: global south scholars and their tensions in border-crossing to global north journals. *BMJ Glob Health*. 2024;9(4):e014420. doi:10.1136/bmjgh-2023-014420.
10. Naidu T. Modern medicine is a colonial artifact: introducing decoloniality to medical education research. *Acad Med*. 2021;96(11S):S9–S12. doi:10.1097/ACM.0000000000004339.
11. Grosfoguel R. Decolonizing post-colonial studies and paradigms of political-economy: transmodernity, decolonial thinking, and global coloniality. *TRANSMODERNITY: J Peripheral Cult Prod Luso-Hispanic World*. 2011;1(1):1–36.
12. Bleakley A, Brice J, Bligh J. Thinking the post-colonial in medical education. *Med Educ*. 2008;42(3):266–270. doi:10.1111/j.1365-2923.2007.02991.x.

13. Chow CJ, Hirshfield LE, Wyatt TR. Sharpening our tools: conducting medical education research using critical theory. *Teach Learn Med.* 2022;34(3):285–294. doi:[10.1080/10401334.2021.1946401](https://doi.org/10.1080/10401334.2021.1946401).
14. Paton M, Naidu T, Wyatt TR, et al. Dismantling the master's house: new ways of knowing for equity and social justice in health professions education. *Adv Health Sci Educ Theory Pract.* 2020;25(5):1107–1126. doi:[10.1007/s10459-020-10006-x](https://doi.org/10.1007/s10459-020-10006-x).
15. Wood CG, Bauer HH. Dogmatism in science and medicine: how dominant theories monopolize research and stifle the search for truth. *CHOICE: Curr Rev Acad Libr.* 2012;50(3):499.
16. Fineout-Overholt E, Melnyk BM, Schultz A. Transforming health care from the inside out: advancing evidence-based practice in the 21st century. *J Prof Nurs.* 2005;21(6):335–344. doi:[10.1016/j.profnurs.2005.10.005](https://doi.org/10.1016/j.profnurs.2005.10.005).
17. Wade DT, Halligan PW. The biopsychosocial model of illness: a model whose time has come. *Clin Rehabil.* 2017;31(8):995–1004. doi:[10.1177/0269215517709890](https://doi.org/10.1177/0269215517709890).
18. Rashid MA, Griffin A. Is west really best? The discourse of modernisation in global medical school regulation policy. *Teach Learn Med.* 2024;36(4):504–515.
19. Kinsella EA, Whiteford GE. Knowledge generation and utilisation in occupational therapy: towards epistemic reflexivity. *Aust Occup Ther J.* 2009;56(4):249–258. doi:[10.1111/j.1440-1630.2007.00726.x](https://doi.org/10.1111/j.1440-1630.2007.00726.x).
20. Finn GM, Brown MEL. When 'best' practice isn't. *Med Sci Educ.* 2024;34(4):915–917. doi:[10.1007/s40670-024-02048-2](https://doi.org/10.1007/s40670-024-02048-2).
21. Jamieson MK, Govaart GH, Pownall M. Reflexivity in quantitative research: a rationale and beginner's guide. *Soc Pers Psych.* 2023;17(4):1–15. doi:[10.1111/spc3.12735](https://doi.org/10.1111/spc3.12735).
22. Olmos-Vega FM, Stalmeijer RE, Varpio L, et al. A practical guide to reflexivity in qualitative research: AMEE Guide No. 149. *Med Teach.* 2022;45(3):1–11. doi:[10.1080/0142159X.2022.2057287](https://doi.org/10.1080/0142159X.2022.2057287).
23. Ramani S, Leinster S. AMEE Guide no. 34: teaching in the clinical environment. *Med Teach.* 2008;30(4):347–364. doi:[10.1080/01421590802061613](https://doi.org/10.1080/01421590802061613).
24. Comaroff J, Comaroff JL. Theory from the south: or, how Euro-America is evolving toward Africa. *Anthropol Forum.* 2012;22(2):113–131. doi:[10.1080/00664677.2012.694169](https://doi.org/10.1080/00664677.2012.694169).
25. Magill KR. Identity, consciousness, and agency: critically reflexive social studies praxis and the social relations of teaching. *Teach Teach Edu.* 2021;104:103382. doi:[10.1016/j.tate.2021.103382](https://doi.org/10.1016/j.tate.2021.103382).
26. Freire P. *Pedagogy of the Oppressed*. 30th Anniversary ed. 2000. New York, United States of America: The Continuum International Publishing Group Inc.
27. Ng SL, Rowland P, Kinsella EA. Emancipatory knowledge and epistemic reflexivity: the knowledge and practice for change? *Med Educ.* 2021;55(1):8–10. doi:[10.1111/medu.14414](https://doi.org/10.1111/medu.14414).
28. Ellaway RH, Wyatt TR. When I say resistance. *Med Educ.* 2022;56(10):970–971. doi:[10.1111/medu.14870](https://doi.org/10.1111/medu.14870).
29. Thambinathan V, Kinsella EA. When I say ... anti-racist praxis. *Med Educ.* 2023;57(6):511–513. doi:[10.1111/medu.14997](https://doi.org/10.1111/medu.14997).
30. Sims DA, Naidu T. How to ... do decolonial research. *Clin Teach.* 2024;21(6):e13806.
31. Paradis E, Nimmon L, Wondimagegn D, et al. Critical theory: broadening our thinking to explore the structural factors at play in health professions education. *Acad Med.* 2020;95(6):842–845. doi:[10.1097/ACM.0000000000003108](https://doi.org/10.1097/ACM.0000000000003108).
32. Kincheloe JL, McLaren P. Rethinking Critical Theory and Qualitative Research, in *Key Works in Critical Pedagogy*. 2011:285–326.
33. Bishop E, Shepherd M. Ethical reflections: examining reflexivity through the narrative paradigm. *Qual Health Res.* 2011;21(9):1283–1294. doi:[10.1177/1049732311405800](https://doi.org/10.1177/1049732311405800).
34. Guillemin M, Gillam L. Ethics, reflexivity, and "ethically important moments" in research. *Qual Inq.* 2004;10(2):261–280. doi:[10.1177/1077800403262360](https://doi.org/10.1177/1077800403262360).
35. Nixon SA. The coin model of privilege and critical allyship: implications for health. *BMC Public Health.* 2019;19(1):1637.
36. Verdonk P. When I say ... reflexivity. *Med Educ.* 2015;49(2):147–148. doi:[10.1111/medu.12534](https://doi.org/10.1111/medu.12534).
37. Gani JK, Khan RM. Positionality statements as a function of coloniality: interrogating reflexive methodologies. *Int Stud Q.* 2024;68(2):1–13. doi:[10.1093/isq/sqae038](https://doi.org/10.1093/isq/sqae038).
38. Thambinathan V, Kinsella EA. Decolonizing methodologies in qualitative research: creating spaces for transformative praxis. *Int J Qual Methods.* 2021;20:1–9. doi:[10.1177/16094069211014766](https://doi.org/10.1177/16094069211014766).
39. Watt D. On becoming a qualitative researcher: the value of reflexivity. *TQR.* 2015;12(1):82–101. doi:[10.46743/2160-3715/2007.1645](https://doi.org/10.46743/2160-3715/2007.1645).
40. Hafferty FW, Franks R. The hidden curriculum, ethics teaching, and the structure of medical education. *Acad Med.* 1994;69(11):861–871. doi:[10.1097/00001888-199411000-00001](https://doi.org/10.1097/00001888-199411000-00001).
41. Martimianakis MAT, Michalec B, Lam J, et al. Humanism, the hidden curriculum, and educational reform: a scoping review and thematic analysis. *Acad Med.* 2015;90(11 Suppl):S5–S13. doi:[10.1097/ACM.0000000000000894](https://doi.org/10.1097/ACM.0000000000000894).
42. Chuang AW, et al. To the point: reviews in medical education-taking control of the hidden curriculum. *Am J Obstet Gynecol.* 2010;203(4):316 e1–6.
43. Hosseini A, Ghasemi E, Nasrabadi AN, et al. Strategies to improve hidden curriculum in nursing and medical education: a scoping review. *BMC Med Educ.* 2023;23(1):658. doi:[10.1186/s12909-023-04652-z](https://doi.org/10.1186/s12909-023-04652-z).
44. Ewen S, Mazel O, Knoche D. Exposing the hidden curriculum influencing medical education on the health of Indigenous people in Australia and New Zealand: the role of the critical reflection Tool. *Acad Med.* 2012;87(2):200–205. doi:[10.1097/ACM.0b013e31823fd777](https://doi.org/10.1097/ACM.0b013e31823fd777).
45. Ackerman-Barger K, London M, White D. When an omitted curriculum becomes a hidden curriculum: let's teach to promote health equity. *J Health Care Poor Underserved.* 2020;31(4S):182–192. doi:[10.1353/hpu.2020.0149](https://doi.org/10.1353/hpu.2020.0149).
46. Jain NR, Nimmon L, Bulk LY. How to ... bring a JEDI (justice, equity, diversity and inclusion) lens to your research. *Clin Teach.* 2024;21(1):e13660.
47. Wyatt TR. "The sins of our forefathers": reimaging research in health professions education. *Adv Health Sci Educ Theory Pract.* 2022;27(4):1195–1206. doi:[10.1007/s10459-022-10111-z](https://doi.org/10.1007/s10459-022-10111-z).

48. Timmis S, Mgwashu E, Trahar S, et al. Students as co-researchers: participatory methods for decolonising research in teaching and learning in higher education. *Teach Higher Edu.* 2024;29(7):1793–1812. doi:[10.1080/13562517.2024.2359738](https://doi.org/10.1080/13562517.2024.2359738).
49. Finlay L. "Outing" the researcher: the provenance, process, and practice of reflexivity. *Qual Health Res.* 2002;12(4):531–545. doi:[10.1177/104973202129120052](https://doi.org/10.1177/104973202129120052).
50. Ramani S, Könings KD, Mann K, et al. A guide to reflexivity for qualitative researchers in education. *Acad Med.* 2018;93(8):1257–1257. doi:[10.1097/ACM.0000000000002263](https://doi.org/10.1097/ACM.0000000000002263).
51. Koum Besson ES. How to identify epistemic injustice in global health research funding practices: a decolonial guide. *BMJ Glob Health.* 2022;7(4):e008950. doi:[10.1136/bmjgh-2022-008950](https://doi.org/10.1136/bmjgh-2022-008950).
52. Rashid MA, Khan AA. Respect and reflexivity: international education partnerships in primary care. *Educ Prim Care.* 2023;34(3):119–122. doi:[10.1080/14739879.2023.2178332](https://doi.org/10.1080/14739879.2023.2178332).
53. Verdonk P, Abma T. Intersectionality and reflexivity in medical education research. *Med Educ.* 2013;47(8):754–756. doi:[10.1111/medu.12258](https://doi.org/10.1111/medu.12258).
54. Baum F, MacDougall C, Smith D. Participatory action research. *J Epidemiol Community Health.* 2006;60(10):854–857. doi:[10.1136/jech.2004.028662](https://doi.org/10.1136/jech.2004.028662).
55. McKivett A, Paul D. Recreating the future-Indigenous research paradigms in health professional education research. *Med Educ.* 2024;58(1):149–156. doi:[10.1111/medu.15154](https://doi.org/10.1111/medu.15154).
56. Greitemeyer T, Osswald S, Fischer P, et al. Civil courage: implicit theories, related concepts, and measurement. *J Posit Psychol.* 2007;2(2):115–119. doi:[10.1080/17439760701228789](https://doi.org/10.1080/17439760701228789).